

St James's Hospital — Respiratory Outpatient Department

Date: 22 Aug 2024

CLINIC LETTER

Patient: Lauren Bennett

DOB: 1991-04-18 (age 33)

NHS Number: 999 900 0009

Dear Dr Choudhury,

Thank you for referring Ms Bennett for respiratory review. I saw her today following her recent symptom escalation.

History: Childhood-onset asthma, previously well-controlled on salbutamol PRN. Over the past 4 months symptoms have escalated. She now reports salbutamol use 4-5 days per week, nocturnal waking with cough 2 nights per week, and reduced exercise tolerance. No identified new triggers (no pets, no smoking, no occupational exposure). ACT score has fallen to 15/25.

PEFR diary (2 weeks): Mean morning PEFR 280 L/min, mean evening 360 L/min, diurnal variability 22% (significant variability >20% is indicative of poorly controlled asthma).

Examination: RR 18, SpO₂ 97% on air. Chest: occasional end-expiratory wheeze, no crackles. No accessory muscle use at rest.

Spirometry (today): FEV₁ 2.18 L (72% predicted), FVC 3.20 L (94% predicted), FEV₁/FVC 0.68 (reduced, consistent with obstructive defect). 12% reversibility post-bronchodilator.

Allergies: NKDA. Atopy with seasonal allergic rhinitis.

Assessment: Symptom pattern, ACT score, PEFR variability, and spirometry are consistent with **moderate persistent asthma** (GINA classification, step 3). Patient previously on reliever-only therapy which is inadequate given current control. Requires initiation of inhaled corticosteroid as preventer.

Plan:

- **Start beclometasone dipropionate 200 mcg MDI BD** (Clenil Modulite)
- Continue salbutamol 100 mcg MDI as reliever PRN
- Updated asthma action plan provided; education on preventer-vs-reliever distinction
- Inhaler technique reviewed - good MDI technique with spacer recommended
- PEFR diary to continue for 8 weeks
- Respiratory clinic follow-up at 12 weeks to assess response
- If poor response, consider step-up to ICS/LABA combination

Yours sincerely,

Dr M. Singh, Consultant Respiratory Physician